

Guidance and surveillance capability for bystander *Staphylococcus aureus* tetracycline resistance under doxy-PEP: a three-stream evidence audit

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Running title: Surveillance and doxy-PEP bystander *S. aureus* resistance

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Synopsis

Background. Doxycycline post-exposure prophylaxis (doxy-PEP) prevents bacterial sexually transmitted infections and has scaled into guidance internationally. Doxycycline also selects tetracycline resistance beyond its target, and *Staphylococcus aureus*—a commensal and pathogen for which doxycycline is one of a limited number of oral options—is the natural bystander.

Objectives. To determine whether the guidance and surveillance deployed around doxy-PEP could follow the bystander-resistance signal the pivotal trial's final analysis resolved, rather than to estimate a population effect.

Methods. A reproducible, public-data audit of three evidence streams—clinical trials, governmental guidance and deployed US surveillance—each scored against one yardstick: the within-exposed relative risk it could detect, benchmarked to an empirical colonisation estimate.

Results. The final randomised DoxyPEP analysis reported a 3.89-fold higher hazard of incident doxycycline-resistant *S. aureus* under doxy-PEP (95% CI 1.42–10.68; 68 of 393 versus 5 of 163 incident events), with no effect on colonisation clearance. Yet 0 of 6 outbreak-matched governmental guidelines (0 of 10 coded) required *S. aureus* monitoring, and 0 of 12 established

US surveillance systems linked doxy-PEP exposure to an *S. aureus* tetracycline phenotype at a common population denominator. Under realistic exposure density and isolate volumes, an ordinary population system would need an implausible within-exposed effect ($RR \approx 6\text{--}28$) to move a rate.

Conclusions. The upstream trial generated information the downstream architecture is not built to receive. Following the signal requires sentinel surveillance linking doxy-PEP exposure to the *S. aureus* tetracycline phenotype at a common denominator. No population-level causal claim is made.

Introduction

Doxycycline post-exposure prophylaxis (doxy-PEP) substantially reduces bacterial sexually transmitted infections (STIs) and has moved quickly from trial result into guidance at every level, from the World Health Organization through national and municipal health departments.^{1,2} Every dose is selective pressure, and not only on the organisms it targets. *Staphylococcus aureus* is the consequential bystander: it colonises the anatomical sites these trials swabbed and carries mobile tetracycline resistance determinants.³ For two decades, community MRSA in these networks has been documented not as steady-state carriage but as clustered, outbreak-pattern transmission—the USA300 lineage moving through sexual networks in San Francisco, Tokyo and Chicago^{4,5}; the organism colonising these networks is the same one carried on skin community-wide, so resistance selected within the network could, if transmitted, extend beyond it. Tetracycline-resistant USA300 was in fact documented in an MSM-predominant community health population two decades ago, although by the doxycycline-sparing *tet(K)* mechanism, with those isolates remaining doxycycline-susceptible.⁶ Doxycycline is one of a limited number of oral options for outpatient skin and soft-tissue infection, a choice often made empirically before any methicillin result returns,^{7,8} so tetracycline susceptibility can matter clinically before methicillin status is known.

Clinicians who prescribe doxy-PEP have registered the concern directly: in a survey of Italian STI-clinic dermatologists, 91.7% named methicillin-resistant *S. aureus* (MRSA) selection

as a worry, tied with gonococcal resistance as the highest-rated,⁹ and a systematic review of 1840 healthcare professionals across four countries likewise placed antimicrobial resistance among the leading concerns about doxy-PEP.¹⁰ The trial literature has now answered a version of it. In the final DoxyPEP analysis, among participants free of doxycycline-resistant *S. aureus* at baseline, doxy-PEP carried a 3.89-fold higher hazard of incident doxycycline-resistant *S. aureus* (95% CI 1.42–10.68; 68/393 versus 5/163; $P = 0.0044$), while the hazard of clearing colonisation did not differ between arms (HR 1.01, 0.69–1.46).¹¹ The trial’s authors called for public-health surveillance of antimicrobial resistance (AMR) in *S. aureus* and other commensals.

That call defines the present question. We ask not whether doxy-PEP drives population-level resistance—a claim we do not make—but whether the guidance and surveillance deployed around doxy-PEP could follow the signal the trial resolved, at the population level where the clinical stake lies. The clinically decisive axis is tetracycline (doxycycline) susceptibility used in empirical oral therapy, not the methicillin axis around which staphylococcal surveillance is organised. We audit the three places an answer would have to come from—the trials, the guidance and the deployed surveillance—each against a single detectability yardstick.

Materials and methods

Design

The study is a documentary and quasi-computational audit assembled in three streams, collected and analysed in sequence: clinical trials (Stream A), the governmental guidance built on them (Stream B) and the population surveillance meant to monitor them (Stream C). No individual-level or restricted data were used; every input is public, so no ethical approval was required. Sources were located, read, coded with page/section/table locators and SHA-256-pinned before analysis; analysis decisions that could go either way were date-logged before results were known; and every figure and table regenerates from a single command over a tested pipeline archived with a citable DOI.¹² A large language model assisted with writing and analysis engineering; the author verified every numerical value and quotation against primary

sources and takes full responsibility (see Acknowledgements).

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Detectability yardstick

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Each stream is scored on one commensurable quantity: the within-exposed relative risk (RR) it could detect. The empirical benchmark is Soge *et al.*¹³ (tetracycline-resistant *S. aureus* colonisation, 18% versus 8% in doxy-PEP users with >3 doses/month versus non-users; $RR \approx 2.25$); a cross-organism gonococcal $RR \approx 1.42$ is carried as a flagged, more conservative secondary. Because the randomised trial reports a within-exposed hazard ratio of 3.89, well above this band, the detectability verdict below is invariant to which anchor is used: the relative risk a population system would need to see (RR 6–28, Results) exceeds both the cross-sectional benchmark and the trial’s own hazard ratio.

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Sources and coding

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The author identified, retrieved and screened all source documents for eligibility; each eligible document was then coded from full text against a prespecified schema, every value carrying a page, section or table locator. For the guidance corpus (Stream B), a first LLM pass coded all 10 documents against the schema; a second, blinded LLM pass—a different interface (chat rather than the command-line agent) but the same model family—independently re-coded a 5-document subsample (25 field-codings) for reliability, and disagreements were resolved by the author against the source. Per-field agreement is reported as raw concordance, which measures the reproducibility of the coding protocol rather than its accuracy; because the passes share a model family their errors are not independent, so a Cohen’s κ is given for context only (Limitations). The surveillance enumeration (Stream C) was coded by a single pass; its inclusion and exclusion list, with a stated reason per system, is given in the Supplementary data and reflects systems as deployed as of August 2026. All coding prompts and outputs are in the public repository.

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Stream A—trials

For each primary-trial *S. aureus* arm comparison (first-party published data only), we computed, from arm sizes and observed control rate, the exact (Fisher) power to detect the externally fixed benchmark effect and the minimum detectable RR at 80% power; fixing the effect externally makes this a design-based sensitivity analysis rather than observed power. The *S. aureus* assay is doxycycline throughout (E-test, MIC $\geq$ 16 mg/L). We tabulated how the resistance result was reported across trial venues and secondary syntheses (numerator, denominator basis, conditioning set and terminology). The final trial performed the appropriate participant-level time-to-event test itself; we cite that randomised result directly and did not re-estimate it. Cross-sectional carriage cannot resolve the clustered transmission structure of *S. aureus* in these networks; the supporting dispersion, trend-test degradation and outbreak-shape analyses are MRSA-scoped (MRSA being a subset, not the analytic unit) and are reported in the Supplementary data (available as Supplementary data at JAC Online).

Stream B—guidance

To make the denominator principled, we anchored it to the same clustering evidence used in Stream A: US jurisdictions with documented community-associated MRSA/USA300 transmission in men who have sex with men.^{4,5} We took governmental public-health guidance only (clinic and provider-organisation documents excluded, with reasons). Each document was coded from full text, one record per document, every finding carrying a locator, for whether it requires, suggests or omits *S. aureus* monitoring and whether it names, counsels on or measures the organism (coding provenance above). An independence check separated genuinely independent declines from documents that defer to, or are adapted from, one another.

Stream C—surveillance

We examined a prespecified set of 12 established US surveillance and data systems (federal, state or metropolitan) plausibly capable of capturing, at the required level, either doxy-PEP/doxycycline exposure or *S. aureus* AMR phenotype, and coded each on three capabil-

ities: exposure-side capture, phenotype-side capture (of the *S. aureus* tetracycline unit, not the MRSA subset) and the join, recording the geographic resolution of each side. A supporting analysis asked whether, even at a common denominator, the exposed subgroup is dense enough to move a population rate. The exposed population is indexed on the combined PrEP and people-living-with-HIV male count (primary specification; the trial enrolled both, AIDSVu 2024 state PrEP and prevalence), so the dilution fraction is $f = \phi((\text{male-PrEP} + \text{male-PLWH}) \text{ density} / 10^5)$ uptake $d \kappa$, where d is the share of users taking >3 doses/month (the subgroup carrying the benchmark effect) and κ the isolate-enrichment factor; f is population-independent because it tracks exposure density, not headcount, and the minimum detectable change in a proportion near baseline R_0 inverts to $\text{RR}_{\text{needed}} = 1 + \text{MDE} / (f R_0)$. Parameters were not uniformly generous: isolate volume was set generously high (favouring detection), while the realistic under-sampling ($\kappa = 0.5$) and dose-threshold ($d = 0.5$) factors bound against it. Both ranges and the resulting $\text{RR}_{\text{needed}}$ are reported; full grids and the metropolitan break-even are in the Supplementary data.

Results

Stream A: trial measurement and reporting of the *S. aureus* endpoint

When the analysis matched the question, the signal appeared. The final DoxyPEP analysis ran the participant-level test the interim had not—time to first detection of doxycycline-resistant *S. aureus* by arm, standard-of-care censored at crossover, by Cox proportional hazards—and returned HR 3.89 (95% CI 1.42–10.68; $P = 0.0044$), with colonisation clearance unchanged (HR 1.01).¹¹ The 2023 interim, by contrast, was presented as reassuring on a cross-sectional resistance fraction and was underpowered for any externally specified effect (0 of 4 comparisons powered; minimum detectable RR 3.8–5.1 against the benchmark 1.42–2.25). The hazard ratio is not comparable to that cross-sectional minimum: the estimand, not the sample size alone, became adequate to the question.

Two measurement features of the interim record persist and matter downstream. First, a unit mismatch: the trial’s data table is headed “phenotypic resistance to the tetracycline antibiotic

class” while its methods define the *S. aureus* assay as doxycycline, and the federal guideline 124
then describes the doxycycline result as “tetracycline resistance in *S. aureus*”.² Because ef- 125
flux and ribosomal-protection mechanisms differ in whether doxycycline still works (below), 126
an endpoint scored as “the tetracycline class” does not answer the treatment-relevant question. 127
Second, the signal was reported over differing, incompletely documented denominators and 128
then propagated into secondary reporting under further contrasts (Table 1): the resistant nu- 129
merator changes from 5 to 16 to 28 and the denominator basis shifts from colonised carriers to 130
all-swabbed, while the percentage travels more reliably than the denominator, contrast and ter- 131
minology needed to interpret it. The individual-level data that could reconcile these first-party 132
reports are not publicly available.¹ Even the intensively surveilled gonococcus was phenotyped 133
in fewer than one diagnosis in five (resistance testing unavailable for 83%, 212/256).¹⁴ 134

Across trials the phenotype was measured on incompatible terms: DoxyPEP measured doxy- 135
cycline resistance by E-test over two denominators; DuDHS measured it by disc diffusion in 136
single-digit carriers; DOXYVAC measured only MRSA carriage—a methicillin, not a tetracy- 137
cline, quantity, and one that itself rose within the doxy-PEP arm (1.8% to 9.9%) while being 138
reported as a between-arms null.¹⁵ A pooled estimate is not defensible, and the heterogeneity is 139
itself a finding. The trial resolved the doxycycline phenotype but not its molecular basis—efflux 140
(*tet(K)*) versus ribosomal protection (*tet(M)*)—which a single breakpoint cannot separate.^{3,8} 141

The estimand shift did not propagate downstream: no guidance document in the corpus 142
cites the final time-to-event analysis or any hazard ratio, and the two documents revised after 143
its online publication in March 2025—WHO (2026) and San Francisco (May 2026)—retained a 144
cross-sectional or generic frame. Non-citation is telling here because this corpus demonstrably 145
transmits numbers when it has them: CDC carried the cross-sectional 5%→13% figures and 146
WHO cites Soge’s colonisation estimate.¹³ 147

Stream B: *S. aureus* monitoring requirements in governmental guidance

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The governmental guidance shows a consistent within-document asymmetry: the efficacy ques- 149
tion received a graded, systematic evidence review, while the AMR-harm question—explicitly 150
including resistant-pathogen development—received an ungraded review in the same docu- 151

ment. Of the six documented-outbreak jurisdictions with governmental doxy-PEP guidance (San Francisco, Chicago, New York City, Boston/Massachusetts, Los Angeles County and San Diego), 0 require or suggest *S. aureus* monitoring. The decline is not an artefact of copying: four jurisdictions are independently authored, one (Los Angeles County) is adapted from San Francisco and one (Massachusetts) from an external template. Nor is it for want of naming the organism—three of the six name *S. aureus* specifically, two invoke only a generic “microbiome” risk and one is silent; even where named, none measure it. Across the full coded corpus (10 documents: CDC, WHO and eight US jurisdictions) the result is identical: 0 instrument the bystander. Double-coding covered a subsample of 5 of the 10 documents (25 field-codings across five fields, chosen to span all *S. aureus*-location categories); raw agreement was 92% (23/25), and the single field with meaningful disagreement (bystander treatment; $\kappa = 0.58$, context only) is reported unrounded. Providers are nonetheless required to counsel patients about resistance in commensal organisms—a harm no system measures.

Stream C: surveillance capability and exposure–phenotype linkage

Following the signal requires one system to carry, at a common population denominator, both who is exposed to doxy-PEP and what tetracycline phenotype *S. aureus* carries. Across this prespecified set of twelve systems, none does (Figure 1). Two systems capture exposure at a population denominator (AIDSVu and CDC AtlasPlus, as a pre-exposure prophylaxis proxy, not doxy-PEP itself),¹⁸ and zero capture *S. aureus* tetracycline resistance at a population denominator: the join is vacuous because one half does not exist.

The absence is a codified seam. Wherever *S. aureus* is surveilled at a population denominator the reported axis is methicillin: the one population-based platform, Active Bacterial Core surveillance/Emerging Infections Program (ABCs/EIP), reports invasive MRSA; the National Healthcare Safety Network reports MRSA events; and the only routine source of *S. aureus* tetracycline susceptibility, the facility cumulative antibiogram, is non-standardised and carries no population denominator, because CLSI M39 mandates the MRSA/MSSA split and makes tetracycline a selectively suppressible supplemental agent. The National Antimicrobial Resistance Monitoring System excludes *S. aureus* altogether; tetracycline-resistant *S. aureus* is

nationally notifiable nowhere. Exposure exists at county/metro resolution and phenotype only at denominator-free facility level, so no geography carries both. A system could be built—most directly by adding *S. aureus* tetracycline susceptibility to ABCs/EIP, which already holds the denominator—but none is deployed.

Nor could an ordinary population system recover the missing side by collecting more isolates. Indexing the exposed population on the combined PrEP and people-living-with-HIV male count—the primary specification, because the trial enrolled both (Materials and methods)—the realistic-cell within-exposed effect required to move a population rate runs to $RR \approx 6\text{--}28$ across the design-effect range (Figure 2); the more restrictive PrEP-only indexing gives $RR \approx 14\text{--}64$, which overstates infeasibility. Either way it is far above any plausible effect. Detection appears in a single cell of the entire grid, where every generous assumption is maxed at once (55% uptake, $5\times$ enrichment, 13% baseline, 100 000 isolates/geography-year and optimistic panel independence): there RR_{needed} falls to 1.03, and under single-comparison inference even the most favourable cell stays above the yardstick (1.79). The verdict thus turns on parameters no deployed system reports—exposure density, isolate enrichment and dosing frequency—which is the measurement gap in miniature: the metropolitan best cell needs about twice ($\approx 2\times$) the combined male exposure density of the densest US geography, and realistic cells hundreds of times it. The only configuration that restores sensitivity is to sample *S. aureus* directly from doxy-PEP-exposed persons, which converts surveillance into a sentinel cohort.

Alt text: A schematic summarising the 12-system surveillance audit: exposure is captured only as a PrEP proxy and the *S. aureus* tetracycline phenotype at a population denominator nowhere, so no system links exposure to phenotype.

Alt text: A logarithmic relative-risk axis with a shaded benchmark band at 1.42 to 2.25; the trial hazard ratio 3.89 sits above the band, surveillance detectability thresholds of about 6 to 28 (primary combined-denominator specification; 14 to 64 under the more restrictive PrEP-only specification) sit far to the right, and guidance has no threshold.

The three streams are summarised against the yardstick in Table 2; a PubMed literature-coverage analysis of the bystander organism (361 papers, of which few name or measure it) is reported in the Supplementary data (S5).

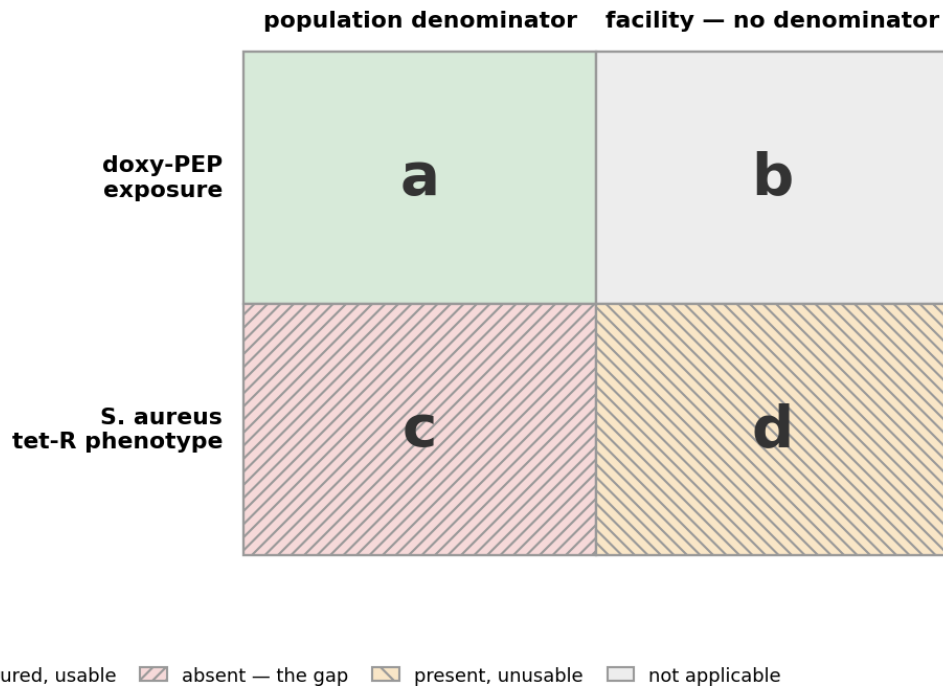


Figure 1: Schematic summary of the 12-system surveillance audit: no deployed US system links doxy-PEP exposure to *S. aureus* tetracycline resistance at a common population denominator. Exposure is captured only as a PrEP proxy (AIDSVu, CDC AtlasPlus); the *S. aureus* tetracycline phenotype is captured at a population denominator nowhere, because the one population-based platform reports the methicillin (MRSA) subset. Of the 12 systems assessed, 0 join exposure to phenotype.

Discussion

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The pivotal trial has now resolved the bystander question it was long thought unable to reach: a significant increase in incident doxycycline-resistant *S. aureus* under randomised exposure, with no matching effect on carriage, and a call from its authors for surveillance of exactly this.¹¹ The upstream instrument generated the information. Across the two systems that would have to receive it—guidance and deployed surveillance—the answer is that they cannot: guidance instruments nothing (0/6; 0/10) and no deployed system joins exposure to the *S. aureus* tetracycline phenotype at a common denominator (0/12).

The sharper reading is that the trial’s measurement system evolved to meet its internal question while the implementation system did not: across 2023–2025 the same endpoint moved from a cross-sectional fraction read as reassurance to a participant-level HR of 3.89, and the guidance and surveillance did not move with it¹⁹—a demonstrated signal handed to systems

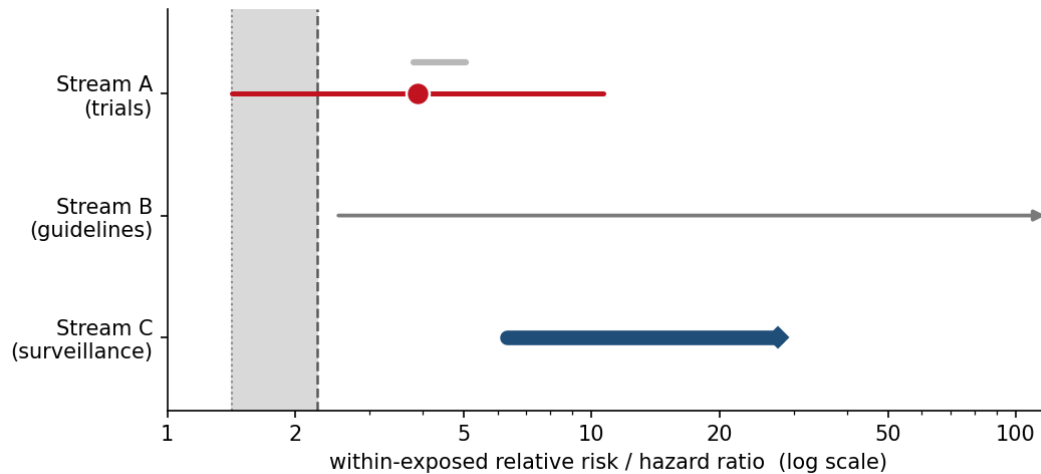


Figure 2: One axis—within-exposed relative risk (log scale)—carries all three streams against the plausible benchmark band (RR 1.42–2.25). The trial’s resolved effect (HR 3.89) clears the band; guidance requires no *S. aureus* measurement so no detection threshold exists; and deployed surveillance would need RR $\approx$ 6–28 to move a population rate (combined PrEP+PLWH exposed denominator; the more restrictive PrEP-only indexing gives 14–64). The trial HR is shown as an empirical magnitude reference only; it is not numerically interchangeable with the surveillance RR threshold because the estimands differ. The signal is above the line while the systems built to follow it are not.

built to measure something else.

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Tetracycline resistance mechanisms and the phenotype unit

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Staphylococcal surveillance is standardised around methicillin, yet the treatment-relevant axis here is tetracycline susceptibility, used empirically before methicillin status is known; the two are orthogonal. So is the molecular basis of tetracycline resistance: *tet*(K) efflux leaves doxycycline largely usable whereas *tet*(M) ribosomal protection can compromise it, and the trial resolved incident doxycycline resistance without resolving which.^{3,8} The distinction is not academic. Of 823 invasive USA300 isolates in national surveillance, 72 (9%) were tetracycline-resistant, yet 69 were doxycycline-susceptible *tet*(K) and only 3 doxycycline-resistant *tet*(M)²⁰—a single breakpoint reporting one number for two clinically opposite situations, on determinants that also differ in mobility (*tet*(M) on conjugative transposons crossing strain boundaries;²¹ *tet*(K) typically plasmid-borne⁶). An exposure cohort shows the same blindness: among 168 military personnel, doxycycline antimalarial prophylaxis was not associated with overall tetracycline-class resistance in *S. aureus*, yet *tet*(M) genes were significantly more common in the exposed

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($p=0.031$)²²—the aggregate phenotype null while the determinant distribution moved (a military, antimalarial cohort, not doxy-PEP). An inherited surveillance variable can stay valid for its original question while being insufficient for the one now imposed on it; we do not forecast which mechanism is spreading, only that the inherited variable cannot say.

Instrument requirements for population monitoring

Following the signal to the population requires an instrument the deployed architecture does not provide: sentinel sampling of *S. aureus* from doxy-PEP-exposed individuals, which raises the exposed fraction by construction and fixes the denominator; sampling that includes the oropharyngeal and peri-anal sites, since nares-only screening under-detects colonisation by about 20% in this population²³; resistance reported per carrier and by mechanism (*tet*(M) versus *tet*(K)) rather than as a pooled tetracycline-class rate; dosing frequency rather than prescription status; a feed carrying *S. aureus* tetracycline susceptibility at a population denominator, most directly by adding it to the platform that already tracks invasive MRSA and joining it to exposure on geography; and access to individual-level trial data. None is exotic; none is in place at scale.

These failures share a structure. A guideline or a surveillance system carries in its sampling frame, cadence, coding, denominator and unit the organism and question it was built for. The doxy-PEP apparatus was built to measure gonococcal efficacy; applied to a bystander commensal it inherits a denominator the intervention itself moves, a phenotype label spanning two mechanisms, and a surveillance architecture standardised around methicillin while the tetracycline unit falls orthogonally through it. We call this *measurement inheritance*: a downstream system retains the dimensions—estimand, denominator, unit, cadence, linkage, measurement responsibility—appropriate to the upstream question but not those the downstream question requires, so the question becomes unanswerable not for want of data but for want of an inherited dimension. It is built from familiar parts—construct validity, estimand mismatch, ascertainment and surveillance bias,²⁴ metric substitution of the Goodhart type²⁵—but concerns how such mismatches couple *across* stages of an evidence chain. The downstream gap is therefore not closable by a better analysis of the trial’s data—that analysis has been done, and it is significant—but is a property of the guidance and surveillance instruments.

Doxy-PEP's benefit accrues to the recipient; the tetracycline selection cost falls on people outside the prescribing decision—the outpatient with a skin infection and, at the sharp end, dialysis, surgical and oncology patients for whom an oral anti-staphylococcal agent is no convenience. A negative externality that no deployed instrument can price is not thereby small; it is unweighable. The wider distributive argument—that the two sides are observed on unequal terms, a property we term *distributive observability*—is developed in the Supplementary data (S6).²⁶

Limitations

We make no population-level causal claim and no transmission claim: whether the individual-level signal propagates into a population externality is precisely what no deployed system can determine. The trial's own randomised result is significant and we accept rather than dispute it; our earlier aggregate reconstruction of the interim is retired in its favour. Colonisation is not infection, and the colonisation–infection link is itself unmeasured here. Much of the evidence for how *S. aureus* clusters and moves through these networks exists only for its MRSA subset, so where our clustering analysis relies on MRSA we mark it as such—an instance of the inherited limitation, not an exception to it. Finally, the trial reaches only a roughly twelve-month horizon; persistence, dose–response and mechanism remain open. Our coding carries two acknowledged asymmetries. The guidance corpus was double-coded on a 5-document subsample by two passes run through different interfaces—a command-line agent and a chat interface—but the same model family, so their errors are not independent; the 92% concordance measures the reproducibility of the coding protocol, not its accuracy. The surveillance enumeration was coded by a single pass. Consistent with a falsification standard set in advance, the inheritance claim fails if a doxy-PEP guideline comes to require *S. aureus* monitoring, or if a deployed system links doxy-PEP exposure to an *S. aureus* tetracycline phenotype at a common population denominator.

Conclusions

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The randomised instrument generated a bystander-resistance signal that the guidance and surveillance built around doxy-PEP are not instrumented to follow. The gap is one of measurement architecture, not of missing data, and it is closable—by sentinel surveillance that links doxy-PEP exposure to the *S. aureus* tetracycline phenotype at a common denominator. Until it is built, the population question the trial’s authors posed cannot be run.

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Transparency declarations

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A. C. D. reports former employment at Gilead Sciences (2020–2024) with prior ownership of corporate stock, all divested by 2024; Gilead played no role in this work. A. C. D. is sole owner of Nyx Dynamics, LLC. No other conflicts to declare. All data are public (AIDSVu; published trials and guidelines) and all analysis code is openly available (<https://github.com/Nyx-Dynamics/doxy-PEP-bystander>) and archived at Zenodo (doi:10.5281/zenodo.22725070), regenerating every figure and table by a single command; every coded source is SHA-256-pinned. No restricted-access data were used.

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Table 1: Propagation of the DoxyPEP *S. aureus* resistance signal across reporting venues. Rows are not assumed to be identical estimands; differences in contrast, conditioning set, denominator and terminology are the object of comparison.

Venue	Reported result	Frame / conditioning set
NEJM appendix ¹	5/31 = 16%	Month-12 doxy-PEP arm; resistant among carriers
CROI abstract ¹⁴	16/137 = 12%	Doxy-PEP arm; all-swabbed denominator
CDC guideline ²	28/222 = 13%; summarised 5%→13%	Doxy-PEP arm; guideline frame
JAMA Insights ¹⁶	5% versus 4%	Doxy-PEP versus control; “infected with”
JAMA Synopsis ¹⁷	5%→16%	Within arm, among <i>S. aureus</i> -isolated

Table 2: Headline audit of the doxy-PEP evidence base against one detectability yardstick. Each cell is licensed by the Results above; no composite score is implied.

Stream	Question	Result
A. Trials	Was the bystander signal resolved?	Yes—HR 3.89 (1.42–10.68), incident doxy-R <i>S. aureus</i>
B. Guidance	Is <i>S. aureus</i> monitoring required?	No—0/6 outbreak-matched; 0/10 coded
C. Surveillance	Is exposure linked to phenotype?	No—0/12 systems; $RR_{\text{needed}} \approx 6\text{--}28$ to detect